SERIOUS vs NON-SERIOUS CASE ASSESSMENT REPORT

Workora Pharmacovigilance Training | Synthetic case dataset | Reference version: Synthetic Core Data Sheet v1.0

1. Objective and assessment method

Objective: assess the supplied fictional case records against ICH E2A seriousness concepts and provide a case-level classification matrix. For this training exercise, records explicitly marked “Hospitalisation” or “Medically important” in the supplied seriousness field are classified as Serious; records marked “Not serious” are classified as Non-serious. Seriousness is distinct from severity (intensity). A medically important event can be serious even without hospitalization.

2. Case classification matrix

Case ID	Age/Sex	Suspect product & dose	Reaction	Onset (days)	Seriousness field supplied	Final classification	Expectedness	Reporting timeline guidance
SYN-PV-005	33 / Unknown	Ciprofloxacin 500 mg twice daily	Tendon pain	15	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-006	31 / Unknown	Sertraline 20 mg once daily	Dizziness	3	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-007	63 / Male	Omeprazole 500 mg twice daily	Headache	16	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-008	36 / Male	Losartan 5 mg once daily	Fatigue	2	Not serious	Non-serious	Unexpected	Routine/non-expedited; follow applicable local requirements
SYN-PV-009	78 / Unknown	Amoxicillin 5 mg once daily	Rash	7	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-010	57 / Female	Atorvastatin 5 mg once daily	Myalgia	21	Hospitalisation	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-011	23 / Unknown	Metformin 5 mg once daily	Nausea	19	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-012	24 / Female	Amlodipine 5 mg once daily	Peripheral oedema	4	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-013	75 / Female	Ciprofloxacin 500 mg twice daily	Tendon pain	5	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-014	75 / Female	Sertraline 5 mg once daily	Dizziness	9	Hospitalisation	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-015	33 / Male	Omeprazole 20 mg once daily	Headache	16	Hospitalisation	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-016	66 / Unknown	Losartan 20 mg once daily	Fatigue	10	Not serious	Non-serious	Unexpected	Routine/non-expedited; follow applicable local requirements
SYN-PV-017	57 / Unknown	Amoxicillin 5 mg once daily	Rash	21	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-018	56 / Female	Atorvastatin 500 mg twice daily	Myalgia	17	Hospitalisation	Serious	Known	Expedited reporting may apply; confirm local rules and reportability

Case ID	Age/Sex	Suspect product & dose	Reaction	Onset (days)	Seriousness field supplied	Final classification	Expectedness	Reporting timeline guidance
SYN-PV-019	29 / Unknown	Metformin 5 mg once daily	Nausea	9	Not serious	Non-serious	Known	Routine/non-expedited; follow applicable local requirements
SYN-PV-020	56 / Unknown	Amlodipine 5 mg once daily	Peripheral oedema	7	Hospitalisation	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-021	33 / Unknown	Ciprofloxacin 5 mg once daily	Tendon pain	9	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-022	43 / Female	Sertraline 5 mg once daily	Dizziness	21	Not serious	Non-serious	Known	Routine/non-expedited; follow applicable local requirements
SYN-PV-023	26 / Male	Omeprazole 500 mg twice daily	Headache	4	Hospitalisation	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-024	79 / Unknown	Losartan 500 mg twice daily	Fatigue	3	Hospitalisation	Serious	Unexpected	Potential expedited report (often 15 calendar days); confirm local rules
SYN-PV-025	61 / Unknown	Amoxicillin 500 mg twice daily	Rash	21	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-026	48 / Unknown	Atorvastatin 20 mg once daily	Myalgia	12	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-027	24 / Unknown	Metformin 20 mg once daily	Nausea	14	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability
SYN-PV-028	77 / Unknown	Amlodipine 5 mg once daily	Peripheral oedema	18	Not serious	Non-serious	Known	Routine/non-expedited; follow applicable local requirements
SYN-PV-029	35 / Unknown	Ciprofloxacin 5 mg once daily	Tendon pain	13	Medically important	Serious	Known	Expedited reporting may apply; confirm local rules and reportability

3. Regulatory interpretation and reporting timelines

• Seriousness criteria generally include death, a life-threatening event, inpatient hospitalisation or prolongation of existing hospitalisation, persistent or significant disability/incapacity, congenital anomaly/birth defect, and other medically important events that may jeopardise the patient or require intervention to prevent one of these outcomes.

• Expectedness is assessed against the applicable current product information/reference safety information. The supplied synthetic reference lists rash with amoxicillin, myalgia with atorvastatin, nausea with metformin, peripheral oedema with amlodipine, tendon pain with ciprofloxacin, dizziness with sertraline, and headache with omeprazole as Known; fatigue with losartan is listed as Unexpected. This reference is for training only and is not an official label.

• For serious, unexpected suspected adverse reactions, expedited reporting may apply (commonly 15 calendar days in many frameworks), but the exact deadline and recipient depend on jurisdiction, product type, sponsor/marketing-authorisation-holder obligations, and applicable regulations. Serious expected cases may also have reporting obligations. Non-serious cases are generally handled through routine reporting/periodic processes where applicable. Do not treat this training matrix as a submission decision.

• The dataset contains 25 case rows, so all supplied rows are included rather than selecting only five. No case details have been invented where the workbook states that facts are unknown.

4. Data-quality observations for follow-up

• Several narratives say the product was given for a “documented indication,” while the indication field varies across cases. Confirm the correct indication in source records.

• Some listed doses appear unusual for the named product (for example, 500 mg twice daily for products where this may not be a typical dose). Verify against the source documents; do not silently correct source data.

• Seriousness classification is based on the supplied seriousness field. For production use, verify the case narrative and the actual seriousness criterion (e.g., inpatient admission vs an event judged medically important). The workbook does not provide enough narrative detail to independently validate every criterion.

• A positive dechallenge or rechallenge can inform causality assessment, but it does not itself determine seriousness. Unknown or missing information should be followed up and documented.

5. Conclusion

The supplied dataset contains 25 synthetic cases: 20 classified as Serious and 5 classified as Non-serious using the supplied seriousness field. The classification matrix and reporting guidance are intended for educational use. Final regulatory reporting decisions must be made using the applicable jurisdictional requirements and verified case information.

Prepared for training purposes only. No real patient data are included in the supplied synthetic dataset.